HEALTH BEACON

CMSRN EXAM PREP

Practice Exam 01 - Elements of Interprofessional Care

21 scored practice questions | Printable Practice Exam

Instructions	Select the single best answer for each question. An answer key with explanations begins after the question section.

PART A - QUESTIONS

1.

Scenario

A patient with heart failure and chronic kidney disease is receiving IV furosemide on a medical unit. Before the morning dose, the patient reports feeling faint when standing. Supine blood pressure is 118/66 mm Hg; after standing with assistance, blood pressure is 88/52 mm Hg. Urine output has been 1,900 mL over 8 hours.

Which modification to the nursing plan is most appropriate based on these findings?

A) Return the patient safely to bed, assess volume status, and notify the provider before giving the dose

B) Give the furosemide as scheduled and restrict oral fluids for the rest of the shift

C) Ask physical therapy to evaluate the patient before obtaining any additional assessment data

D) Recheck the standing blood pressure after the patient walks in the hallway

2.

Scenario

During rounds, the speech-language pathologist recommends a minced-and-moist diet for a patient after a stroke. The RN has observed repeated coughing and a wet voice even with that texture. The dietitian is waiting to finalize the meal plan.

How should the RN contribute to the team decision?

A) Enter a nothing-by-mouth order until the next scheduled speech-language pathology visit

B) Report the observed response and request a joint reassessment before additional oral intake

C) Ask the dietitian to choose a different texture without involving the other team members

D) Continue the recommended diet because swallowing recommendations are outside the RN's role

3.

Scenario

A patient hospitalized for a heart failure exacerbation is expected to go home tomorrow. The patient lives alone, has no working scale, depends on a neighbor for transportation, and missed two clinic visits last month. The patient can accurately describe the low-sodium diet and medication schedule.

Which action best addresses the patient's greatest transition risk?

A) Repeat the written heart failure education using larger print

B) Ask the neighbor to assume responsibility for daily symptom monitoring

C) Coordinate access to a scale and reliable follow-up transportation before discharge

D) Recommend postponing discharge until the patient no longer needs outpatient visits

4.

Scenario

When removing oxygen tubing, an RN finds a previously undocumented area of nonblanchable erythema behind the patient's right ear. The skin is intact. The RN removes the pressure source, assesses the area, and notifies the appropriate team members.

Which chart entry best documents the finding?

A) Small pressure sore caused by oxygen tubing; wound team notified

B) Skin behind ear looks red but should improve now that tubing was moved

C) Stage 1 injury, approximately dime-sized, discovered during routine care

D) Intact 1.2-cm area of nonblanchable erythema behind right ear; tubing repositioned and pressure-relief pad applied

5.

Scenario

An RN programs an IV medication using a smart pump. The ordered dose has been verified by pharmacy, but the pump displays a hard-limit alert and will not start the infusion.

What is the safest nursing response?

A) Select a basic infusion mode so the verified medication can be started on time

B) Stop programming and reconcile the order, concentration, pump library, and patient parameters

C) Divide the dose into two sequential infusions that each fall below the hard limit

D) Use a pump from another unit because its drug library may accept the programmed dose

6.

Scenario

A patient with type 2 diabetes is receiving prednisone for an inflammatory disorder. Morning fasting glucose remains near baseline, but pre-dinner readings have risen above 260 mg/dL for two days. The current care plan emphasizes fasting glucose monitoring.

How should the RN best individualize the assessment plan?

A) Trend glucose around meals and relate the pattern to prednisone timing and food intake

B) Continue fasting checks only because they provide the most consistent daily comparison

C) Request a hemoglobin A1c each morning until the prednisone course is completed

D) Delay further assessment until symptoms of hyperglycemia are reported

7.

Scenario

Physical therapy documented that a patient could transfer with one-person assistance and recommended discharge home. Later, assistive personnel report that the patient's knee buckled twice during a transfer. The discharge order has already been entered.

What is the RN's best collaborative response?

A) Cancel the discharge order and independently change the destination to skilled nursing care

B) Proceed with discharge because the therapist's assessment determines mobility readiness

C) Communicate the new finding and arrange reassessment with therapy and the discharge team

D) Ask assistive personnel to repeat the transfer until the patient completes it without buckling

8.

Scenario

A patient with a new colostomy is medically ready for discharge. Arthritis limits hand dexterity, and the patient has difficulty cutting and sealing the pouch. The patient's adult child lives two hours away and cannot provide daily care.

Which plan most directly promotes a safe transition home?

A) Arrange adapted supplies, confirm performance with return demonstration, and coordinate home ostomy support

B) Give written instructions to the adult child and arrange delivery of standard ostomy supplies

C) Extend the admission until the patient can apply the standard pouch without assistance

D) Tell the patient to contact the surgeon's office if leakage occurs after discharge

9.

Scenario

An electronic clinical-decision support tool flags a medical-unit patient as being at high risk for sepsis. The RN has not yet assessed the patient since the alert appeared.

How should the RN use this information?

A) Document sepsis as the patient's diagnosis because the algorithm identified the risk

B) Compare the alert with a focused assessment and current data, then escalate concerning findings

C) Wait for the provider to acknowledge the alert before collecting additional patient information

D) Silence the alert if the previous vital signs were within the expected range

10.

Scenario

A patient with chronic obstructive pulmonary disease is to be discharged with newly prescribed home oxygen. During teaching, the patient states that cigarettes are smoked inside the home and sees no reason to change that practice.

What is the best next step by the RN?

A) Complete discharge because the equipment vendor is responsible for oxygen safety teaching

B) Ask the family to sign a statement accepting responsibility for any oxygen-related fire

C) Alert the prescriber and transition team, address fire risk and readiness, and revise the plan before discharge

D) Tell the patient that home oxygen will be permanently withheld unless everyone stops smoking

11.

Scenario

The laboratory calls an RN with a potassium result of 2.7 mEq/L for a patient receiving a loop diuretic. The patient has new premature ventricular contractions on telemetry. The RN completes a focused assessment.

Which communication best demonstrates closed-loop teamwork?

A) Report the result and rhythm change to the provider, repeat back orders, and confirm the monitoring plan

B) Send the laboratory value in a secure message and assume it will be reviewed during rounds

C) Ask the unit secretary to tell the provider that the patient's potassium is low

D) Document the critical value and wait for the laboratory to contact the provider directly

12.

Scenario

A patient is scheduled for discharge after abdominal surgery and needs daily wound packing. The patient's spouse, who was expected to provide care, becomes pale during teaching and says, “I cannot do this at home.” The patient cannot see or reach the wound.

Which response best supports patient- and family-centered transition planning?

A) Ask the spouse to watch the procedure again because discharge orders are already complete

B) Provide printed steps and reassure the spouse that confidence will improve after discharge

C) Teach the patient to perform the procedure using a mirror despite the stated limitations

D) Explore the spouse's concerns and coordinate another feasible wound-care arrangement before discharge

13.

Scenario

An RN notices that a copied-forward progress note states “right internal jugular central line intact,” although the line was removed the previous day. No medication or treatment was given based on the incorrect statement.

What should the RN do?

A) Delete the original note so future clinicians cannot see the inaccurate statement

B) Correct the record according to policy with a dated clarification and report the safety concern

C) Leave the note unchanged because the error did not result in patient harm

D) Ask the next shift to document that the central line is no longer present

14.

Scenario

At the bedside, the barcode medication administration system repeatedly reports that a medication is assigned to a different patient. The RN notices that the hospitalized patient's wristband is faded and difficult to read.

Which action is appropriate?

A) Stop, verify identity using approved identifiers, replace the wristband, and repeat the scan

B) Override the warning after comparing the room number with the medication label

C) Ask another RN to scan the medication under that RN's login credentials

D) Manually enter the unreadable barcode and administer the medication without rescanning

15.

Scenario

A patient with diabetes is preparing for discharge after treatment with high-dose glucocorticoids. The discharge prescription changes the insulin regimen, but the patient says the copayment is unaffordable and the current insulin supply at home is nearly empty.

Which intervention most effectively reduces the risk of a medication-related readmission?

A) Advise the patient to use the remaining insulin at reduced doses until the follow-up visit

B) Provide the manufacturer's website and ask the patient to apply for assistance after discharge

C) Engage the prescriber, pharmacist, and care coordinator to secure an affordable regimen before discharge

D) Keep the prescribed regimen unchanged and document that cost may affect adherence

16.

Scenario

A patient being treated for pneumonia becomes restless and inattentive overnight. The RN initiates the care-plan interventions of reorientation, sensory aids, pain management, and uninterrupted sleep periods. Four hours later, the patient is pulling at the IV and cannot state the location; oxygen saturation has fallen from 95% to 89% on the same oxygen flow.

How should the RN evaluate and respond to the plan?

A) Continue the plan unchanged because delirium interventions usually require 24 hours to work

B) Add a restraint intervention and reassess the patient at the end of the shift

C) Remove the sensory aids to reduce stimulation and allow uninterrupted sleep

D) Recognize worsening status, reassess for physiologic causes, and escalate the change promptly

17.

Scenario

A patient who receives hemodialysis on Monday, Wednesday, and Friday will transfer to a skilled nursing facility with an IV antibiotic to be administered after dialysis. The transfer is planned for Friday morning before the scheduled dialysis session.

Which coordination step is essential before transfer?

A) Confirm the day's dialysis and antibiotic plan with the dialysis center and receiving facility, then include it in the handoff

B) Omit the antibiotic from the transfer record because the dialysis center will determine whether it is needed

C) Reschedule all future dialysis sessions through the hospital unit after the patient has transferred

D) Send the patient first and ask the receiving facility to resolve the treatment timing on arrival

18.

Scenario

A patient slips while walking to the bathroom with assistance and is lowered to the floor. The RN assesses the patient, notifies the provider, begins ordered monitoring, and completes the organization's safety-event report.

Which information belongs in the medical record?

A) The event report number and the name of the staff member believed to be responsible

B) A statement that the fall was unavoidable because the patient did not follow directions

C) A copy of the entire safety-event report, including the quality-review section

D) Objective event details, assessment findings, notifications, interventions, and patient response

19.

Scenario

A pharmacist tells the RN that a newly ordered antimicrobial dose may be excessive for the patient's current renal function. The prescriber responds, “Give it as written,” without discussing the creatinine trend or the pharmacist's concern. The medication is not time-critical.

What should the RN do next?

A) Administer the dose because the prescriber has accepted responsibility for the order

B) Ask the pharmacist to change the dose independently and send the prescriber a message later

C) Restate the unresolved safety concern and use the escalation pathway until the order is clarified

D) Withhold all scheduled medications until the team reaches agreement about the antimicrobial

20.

Scenario

A patient hospitalized for a chronic obstructive pulmonary disease exacerbation has had two readmissions in six months. The patient is ready for discharge, but the telephone number in the record is disconnected and the patient has previously missed follow-up calls.

Which action best strengthens continuity of care?

A) Document the disconnected number and allow the clinic to contact the patient by mail

B) Confirm a preferred contact method and arrange a follow-up appointment before the patient leaves

C) Give the patient the clinic number and recommend calling if breathing worsens

D) Refer the patient to emergency services for all respiratory concerns after discharge

21.

Scenario

A patient recovering from hip-fracture repair has a goal to walk 50 feet by postoperative day 2. Before the planned therapy session, the patient reports pain of 8 on a 0-to-10 scale and says the session is scheduled during a daily religious observance.

How should the RN apply the nursing process to this situation?

A) Reassess barriers and collaborate with the patient and team to adjust timing and supportive interventions

B) Keep the original schedule because changing a measurable goal makes evaluation unreliable

C) Cancel the mobility goal until the patient can walk without premedication or scheduling changes

D) Document refusal of therapy and ask the provider to revise the discharge date

PART B - ANSWER KEY AND EXPLANATIONS

Question numbering below corresponds to the scored questions in Part A.

1	A	2	B	3	C	4	D	5	B
6	A	7	C	8	A	9	B	10	C
11	A	12	D	13	B	14	A	15	C
16	D	17	A	18	D	19	C	20	B
21	A								

1. Correct: A - Return the patient safely to bed, assess volume status, and notify the provider before giving the dose

Symptomatic orthostatic hypotension after substantial diuresis suggests possible intravascular depletion. Safety, reassessment, and provider collaboration are needed before another diuretic dose; ambulation or automatic administration could worsen hypotension.

2. Correct: B - Report the observed response and request a joint reassessment before additional oral intake

The RN contributes current bedside evidence to the shared plan. Coughing and a wet voice raise concern about swallowing safety, so prompt interdisciplinary reassessment is safer than independently changing orders or ignoring the change.

3. Correct: C - Coordinate access to a scale and reliable follow-up transportation before discharge

The patient understands self-care but lacks the resources required for daily monitoring and follow-up. Addressing these concrete barriers directly reduces transition failure and preventable readmission risk.

4. Correct: D - Intact 1.2-cm area of nonblanchable erythema behind right ear; tubing repositioned and pressure-relief pad applied

Documentation should be objective, measurable, anatomically specific, and include the intervention. The other entries are vague, predictive, or omit important descriptive data.

5. Correct: B - Stop programming and reconcile the order, concentration, pump library, and patient parameters

A hard-limit alert signals a potentially unsafe mismatch that should not be bypassed. The RN should verify all inputs and resolve the discrepancy before infusion rather than using a workaround.

6. Correct: A - Trend glucose around meals and relate the pattern to prednisone timing and food intake

The emerging pattern is later-day hyperglycemia, which fasting-only checks can miss. Targeted trending supplies the data needed to individualize treatment and evaluate the steroid's effect.

7. Correct: C - Communicate the new finding and arrange reassessment with therapy and the discharge team

Knee buckling is new evidence that may change mobility and discharge readiness. The RN should bring it to the relevant disciplines for timely reassessment rather than independently changing disposition or relying on an earlier evaluation.

8. Correct: A - Arrange adapted supplies, confirm performance with return demonstration, and coordinate home ostomy support

This plan matches equipment and services to the patient's dexterity and available support while verifying the patient's ability to manage the pouch. Written instructions or delayed problem solving do not address the functional barrier.

9. Correct: B - Compare the alert with a focused assessment and current data, then escalate concerning findings

Decision support supplements rather than replaces nursing judgment. The RN must validate the signal using current clinical data and act on concerning findings instead of treating the algorithm as a diagnosis or ignoring it.

10. Correct: C - Alert the prescriber and transition team, address fire risk and readiness, and revise the plan before discharge

Smoking around oxygen creates a serious home hazard that must be addressed before transition. Collaborative reassessment can incorporate fire-safety education, smoking-cessation support, and a feasible discharge plan without coercion or abandonment.

11. Correct: A - Report the result and rhythm change to the provider, repeat back orders, and confirm the monitoring plan

Critical hypokalemia with ectopy requires timely, direct communication and confirmation of the response plan. Read-back and shared monitoring expectations close the communication loop.

12. Correct: D - Explore the spouse's concerns and coordinate another feasible wound-care arrangement before discharge

The intended caregiver has stated an inability to perform essential care, and the patient cannot do it independently. Patient- and family-centered planning requires a workable alternative, such as another trained caregiver or home health support, before discharge.

13. Correct: B - Correct the record according to policy with a dated clarification and report the safety concern

The record should be corrected transparently using the organization's amendment process so the original entry remains traceable. Reporting the copy-forward hazard also supports system improvement even though no harm occurred.

14. Correct: A - Stop, verify identity using approved identifiers, replace the wristband, and repeat the scan

A mismatch warning and unreadable wristband require resolution before administration. Room number, borrowed credentials, manual entry, or override would defeat patient-identification safeguards.

15. Correct: C - Engage the prescriber, pharmacist, and care coordinator to secure an affordable regimen before discharge

A regimen the patient cannot obtain is not a safe transition plan. Interprofessional problem solving before discharge can align clinical needs with coverage, assistance programs, or a suitable affordable alternative.

16. Correct: D - Recognize worsening status, reassess for physiologic causes, and escalate the change promptly

Worsening cognition plus declining oxygenation indicates that the plan is not achieving the desired outcome and may reflect physiologic deterioration. The RN should reassess and escalate rather than simply continue environmental measures or add restraints.

17. Correct: A - Confirm the day's dialysis and antibiotic plan with the dialysis center and receiving facility, then include it in the handoff

The timing of dialysis and post-dialysis antimicrobial administration crosses organizations. Explicit confirmation and handoff prevent a missed dialysis session, omitted dose, or incorrectly timed treatment.

18. Correct: D - Objective event details, assessment findings, notifications, interventions, and patient response

The clinical record should describe what occurred and the patient's evaluation and care without blame or reference to protected quality-review material. The separate event report supports organizational review.

19. Correct: C - Restate the unresolved safety concern and use the escalation pathway until the order is clarified

The renal-dose concern remains unresolved, so the RN should advocate for clarification using the chain of communication. Administering despite a credible concern or withholding unrelated medications is unsafe, and the pharmacist should not alter the order independently.

20. Correct: B - Confirm a preferred contact method and arrange a follow-up appointment before the patient leaves

Verifying how the patient can be reached and securing follow-up before discharge closes known continuity gaps. Merely providing a number or relying on a disconnected contact method shifts unresolved transition work to the patient.

21. Correct: A - Reassess barriers and collaborate with the patient and team to adjust timing and supportive interventions

Nursing-process evaluation uses current patient data to adapt the plan while preserving the functional goal. Addressing pain and the patient's observance supports participation and individualized care rather than labeling the situation as refusal.